

Olfen®-75 SR/Retard *Depotabs*®

Antirheumatic agent with anti-inflammatory and antipyretic properties

Composition

Each Olfen-75 SR/Retard Depotab® contains:
Diclofenac sodium 75mg

Properties and actions

Olfen contains the sodium salt of diclofenac, a non-steroidal active substance having pronounced anti-rheumatic, anti-inflammatory, analgesic and anti-pyretic properties.

Inhibition of prostaglandin biosynthesis, which has been demonstrated experimentally, is thought to be important for the mechanism of action. Prostaglandins play an essential role in the development of inflammation, pain and fever. The anti-inflammatory and analgesic properties become clinically evident in the case of rheumatic disorders in that the symptoms, such as rest pain, pain on motion, morning stiffness, swelling of the joints, are significantly improved and functional ability increases. In post-traumatic/post-operative inflammations, diclofenac causes a rapid reduction in spontaneous pain and pain on motion and reduces inflammatory swelling and wound oedema.

For moderate and severe states of pain of a non-rheumatic type, as well, the pronounced analgesic effect was demonstrated in clinical trials. In cases of primary dysmenorrhoea, diclofenac can alleviate pain and reduce the extent of bleeding.

Pharmacokinetics

After oral administration, diclofenac is rapidly and completely absorbed. The plasma concentrations are linearly dose-dependent. Plasma protein binding amounts to more than 97%. Peak plasma concentrations are attained 1-4 hours after taking the enteric-coated Lactab tablets. The therapeutic plasma concentration range is 0.7-2mcg/ml.

Diclofenac undergoes first pass metabolism. Elimination is about 2/3 renal and about 1/3 via the bile in metabolized form (about 90% of the administered dose in 96 hours). The greater amounts of the hydroxyl-glucuronide and sulphated metabolites are biologically inactive. Only about 1% is renally excreted in unchanged form.

The plasma half-life is about 2 hours and is not affected by kidney failure.

On administration of two 75mg Olfen SR per day, the mean maximum plasma concentrations reached in steady state are approximately 489 ± 237ng/ml.

Indications

- Inflammatory and degenerative forms of rheumatism: rheumatoid arthritis, ankylosing spondylitis, osteoarthritis and spondylarthritis;
- Painful syndromes of the vertebral column;
- Acute gout attack;
- Non-rheumatoid inflammatory pain.

Normal doses

75mg SR Depotabs

Adults: The initial daily dose is usually 100 -150mg. In elderly patients, particularly patients with a low body weight, the initial dosage is 75 -150mg daily. A dosage of 100mg daily can be given by adding 1 diclofenac sodium of 25mg daily and a dosage of 150mg by administering one 75mg SR Depotab every morning and evening.

In milder cases and for chronic treatment, one Olfen-75mg SR Depotab daily is generally sufficient. If symptoms are most severe during the night or in the morning, Olfen-75 SR Depotabs should be taken preferably in the evening. The Depotabs are to be taken unchewed with a glass of water before meals.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Contraindications

Olfen must not be used in the presence of peptic ulcers, hypersensitivity (e.g. with asthma attacks, skin reactions, acute rhinitis) to acetylsalicylic acid or other nonsteroid anti-inflammatory agents, severe disorders of liver function or haemopoiesis disorders.

Adverse effects

Gastrointestinal upsets, upper abdominal pain, eructation, nausea, diarrhoea, light-headedness or headaches at the start of treatment. These adverse effects are generally mild and usually regress after a few days.

Hypersensitivity reactions such as skin rash and pruritus, asthma attacks and a tendency to oedema may arise.

Gastrointestinal bleeding and ulceration, hearing disturbances, photosensitivity, haematuria, blood disorders and renal failure were reported in isolated cases.

Adverse effects: Dermatological: Occasional – rashes or skin eruptions, cases of hair loss, bullous eruptions, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), and photosensitivity reactions have been reported.

Precautions

Patients with evidence of peptic ulcers in their history, those with unexplained gastrointestinal disorders, those with liver or kidney damage and those with high blood pressure as well as elderly

patients require attentive medical supervision.

Blood counts are recommended during prolonged treatment with Olfen, as is the case with other NSAIDs there is a risk of coagulation defects.

Note: NSAIDs may mask the signs and symptoms of infection and should be used cautiously in patients with existing infections.

Precaution:

Severe cutaneous reactions, including Stevens-Johnson Syndrome and toxic epidermal necrolysis (Lyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs.

Warnings

Risk of GI ulceration, bleeding and perforation with NSAID therapy.

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports of fatal GI events.

Warnings:

Cardiovascular Thrombotic Events - observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension - NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure - fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events - all NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with

dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions - NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Interactions

The simultaneous use of Olfen with lithium products or digoxin products increases the lithium or digoxin levels in the blood.

In the case of simultaneous treatment with potassium-sparing diuretics, particular monitoring of serum potassium levels is required since Olfen, like other (nonsteroid) antirheumatics, can lead to an increase in potassium values (hyperpotassaemia).

The simultaneous administration of corticoids or other anti-inflammatory agents increases the risk of gastrointestinal bleeding.

An inhibition of the action of diuretics such as furosemide or etacrynic acid is possible, as is a weakening of the action of antihypertensive drugs.

The simultaneous administration of acetylsalicylic acid leads to a decrease in the blood concentration of the active agent diclofenac.

Pregnancy/breastfeeding

In general, Olfen should be used with caution in first and second trimester of pregnancy. Animal studies have shown no risk for the fetus, but no controlled studies in pregnant women are available. Olfen should not be giving during the third trimester owing to the risk of premature closure of the ductus arteriosus and suppression of uterine contractility. Following oral doses of 50mg given at 8-hour intervals, the active substance of diclofenac sodium passes into the breast milk, but in such small quantities that no unwanted effects on infant are likely to occur.

Overdosage

Management of acute poisoning with NSAIDs consists essentially in supportive and symptomatic measures. Overdosage of diclofenac produces no typical clinical pictures. The following therapeutic measures should be taken in cases of overdosage. Absorption should be prevented as soon as possible after the overdosage by

performing gastric lavage and giving activated charcoal. Supportive and symptomatic treatment is indicated for complications such as hypotension, renal failure, convulsions, gastrointestinal irritation, and respiratory depression. It is unlikely that specific measures such as forced diuresis, dialysis, or haemoperfusion are helpful in eliminating NSAIDs owing to their high protein-binding rate and extensive metabolism.

Other information

Note: Medicines should be kept out of the reach of children.

Storage: Do not store above 30°C.

Expiry: The preparation should not be used after the date marked "EXP" on the pack.

Presentation

Olfen-75 SR Depotabs (Pink, flat, bevel edge film-coated tablets with embossing "75" and "SR")

Packing of 10 x 3 Depotab in blister

The information contained here is limited. Further information can be obtained from your doctor or pharmacist

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Manufactured by Acino, Aesch, Basel-Land, Switzerland
for Acino, Liesberg, Basel-Land, Switzerland

